

PH1 Trichocare

Day 1-8

Day 1 – Part 1: What Is Trichology?

- Trichology = the science of hair and scalp health
- Why understanding the scalp is just as important as styling
- What kinds of problems trichologists help with
- Who does what? Trichologist vs dermatologist vs stylist
- Why clients often talk to their hairdresser first

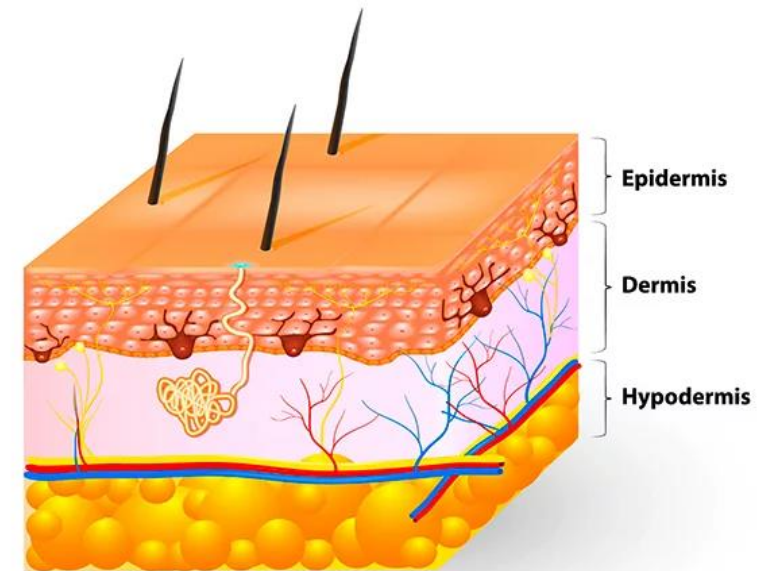


- **Day 1 part 1**
- **Discussion Prompts:**
 - What do you think a trichologist does?
 - Has a client ever asked you about hair loss or flakes?
 - Where would you draw the line between your role and a medical professional?

Day 1 – Part 2: Hair & Scalp Anatomy

- The scalp has 3 layers: epidermis, dermis, and hypodermis
- Hair is made of 3 parts: cuticle (outer), cortex (middle), medulla (core)
- Sebaceous glands release sebum to keep the scalp and hair moisturised
- Hair is mostly protein (keratin) with water, lipids, and pigment
- Understanding this helps with choosing treatments and recognising damage

THE LAYERS OF HUMAN SKIN



- **Day 1 part 2**
- **Discussion Prompts:**
 - Can you feel the difference between healthy and damaged hair?
 - Why do you think the scalp's layers matter for product absorption?
 - What happens when sebum production is too low or too high?

Day 1 – Part 3: Scalp Microbiome & Growth Cycle

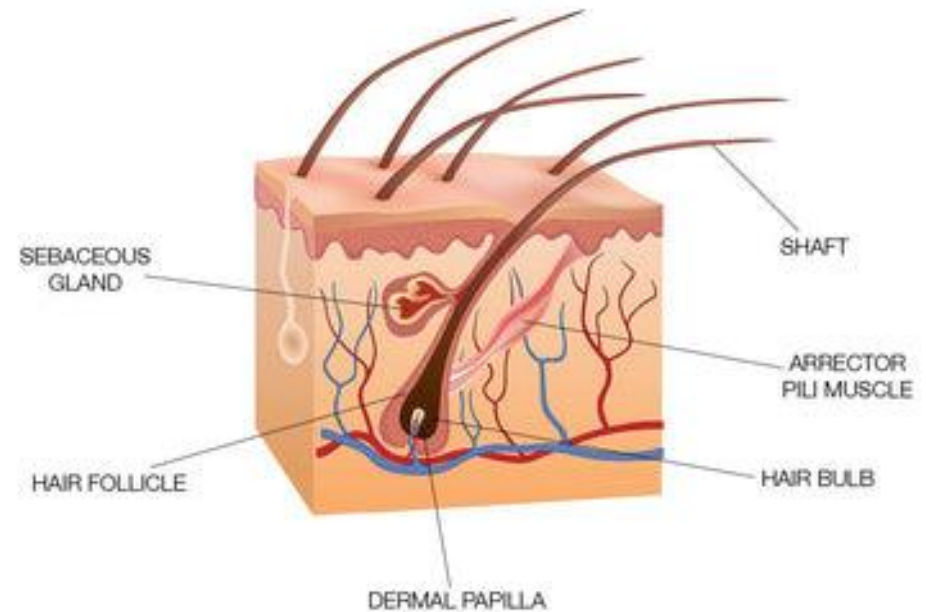
- Hair grows in 4 phases: Anagen (growth), Catagen (transition), Telogen (rest), Exogen (shedding)
- The Anagen (growth) phase can last 2–7 years
- The scalp has its own microbiome — bacteria and yeast that protect the skin barrier
- Product buildup, poor hygiene, or stress can disrupt the balance
- A healthy scalp = better hair retention and reduced shedding



- **Day 1 Part 3**
- **Discussion Prompts:**
 - Why do you think some people shed more hair than others?
 - What kinds of habits might damage the scalp microbiome?
 - Can you spot when a client's hair cycle is out of balance?

Day 2 – Part 1: Hair Growth Phases & Follicle Function

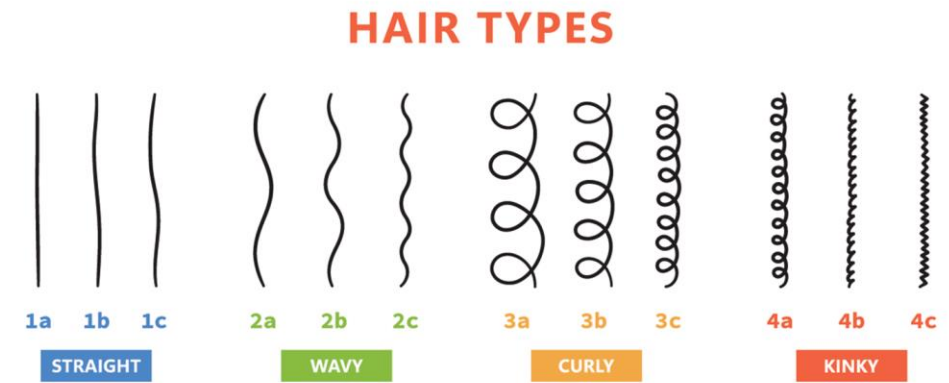
- Hair grows in a cycle: Anagen (growth), Catagen (transition), Telogen (rest), Exogen (shedding)
- Anagen phase lasts 2–7 years — this is where most hair growth happens
- The hair bulb, matrix, and dermal papilla are vital for producing new hair
- Shedding is normal — around 50–100 hairs a day
- Illness, hormones, or stress can affect the cycle or delay regrowth



- **Day 2 Part 1**
- **Discussion Prompts:**
 - What do you think causes hair to stop growing?
 - Have you seen signs of excessive shedding in clients?
 - Why is the follicle so important for healthy regrowth?

Day 2 – Part 2: Hair Texture, Density & Growth Patterns

- Hair texture varies by curl pattern: Type 1 (straight) to Type 4 (coily)
- Afro, Asian, and Caucasian hair differ in shape, thickness, and moisture retention
- Hair density = how many strands per square inch of scalp
- Growth direction patterns include whorls, cowlicks, and natural partings
- Elasticity and diameter affect how hair responds to styling and breakage



- **Day 2 Part 2**
- **Discussion Prompts:**
 - Have you worked with all four hair types?
 - Why does dense hair need different cutting techniques?
 - Can you think of clients whose hair grows in a spiral or cowlick?

Day 2 – Part 3: Hair Damage – Chemical, Physical & Environmental

- Chemical damage: caused by bleach, relaxers, and dyes
- Physical damage: heat tools, rough brushing, and tight styles
- Environmental damage: sun, chlorine, and pollution exposure
- Porosity = how easily hair absorbs and retains moisture
- Elasticity = how much hair can stretch before breaking



- **Day 2 Part 3**
- **Discussion Prompts:**
 - What's the most common type of damage you see in clients?
 - How do you test for porosity or elasticity?
 - How would you explain different types of damage to a client?

Day 3 – Part 1: Dandruff, Dermatitis & Psoriasis

- Dandruff: common, non-contagious, white or grey flaking
- Seborrheic dermatitis: more severe — red, itchy, greasy yellow flakes
- Psoriasis: autoimmune, thick red scaly patches, often hereditary
- Eczema: itchy, dry scalp caused by irritation or allergens
- Pityriasis amiantacea: thick silvery scales that stick to hair shafts



- **Day 3 Part 1**
- **Discussion Prompts:**
 - How do you spot the difference between dandruff and dermatitis?
 - Have you ever seen psoriasis on a client's scalp?
 - When would you refer someone to a dermatologist?

Day 3 – Part 2: Infections, Alopecia Types & Trichotillomania

- Tinea Capitis: fungal infection (ringworm), contagious via combs or towels
- Folliculitis: inflamed follicles, can be bacterial or friction-based
- Alopecia Areata: patchy, autoimmune hair loss
- Traction Alopecia: caused by tight hairstyles and repeated tension
- Trichotillomania: a psychological condition — compulsive hair pulling



- **Day 3 Part 2**
- **Discussion Prompts:**
 - Have you ever seen ringworm or scalp infections?
 - How would you gently discuss traction alopecia with a client?
 - What signs might suggest trichotillomania in a client?



Day 3 – Part 3: Referral Boundaries & Ethical Practice

- Know when not to treat: infections, open wounds, or bleeding
- Language we use - safe, legal language — e.g., “recommend,” not “diagnose”
- Build referral relationships with GPs, dermatologists, and specialists
- Reassure clients calmly: posture, tone, and confidence matter

- **Day 3 Part 3**
- **Discussion Prompts:**
 - Have you ever had to turn a client away for a medical issue?
 - How would you explain a referral without alarming the client?
 - Why is tone and body language important in sensitive cases?

Day 4 – Part 1: Understanding Active Ingredients

- Salicylic acid: exfoliates and reduces flakes — great for dandruff
- Niacinamide: calms inflammation, supports scalp barrier
- Caffeine: stimulates circulation and may support growth
- Peptides & botanicals: nourish and rejuvenate scalp
- Natural oils: olive, shea, castor — protect, soften, and seal moisture



- **Day 4 Part 1**
- **Discussion Prompts:**
 - Have you used products with any of these ingredients?
 - How do you explain 'active ingredients' to a client?
 - Which of these do you think is best for a flaky scalp?

Day 4 – Part 2: Scalp Therapies & In-Salon Protocols



- Steaming: opens follicles and improves absorption of serums and treatments
- Exfoliation: removes build-up and boosts scalp cell turnover
- LED therapy: reduces inflammation and supports healthy growth
- Scalp masks, oils, and serums: tailored to client needs
- Know your contraindications: don't treat open sores, infections, or broken skin

- **Day 4 Part 2**
- **Discussion Prompts:**
 - Have you ever used steam or LED therapy in the salon?
 - What's your go-to scalp treatment for dryness or flakes?
 - When would you avoid treating a client's scalp?

Day 4 – Part 3: Massage for Circulation & Relaxation

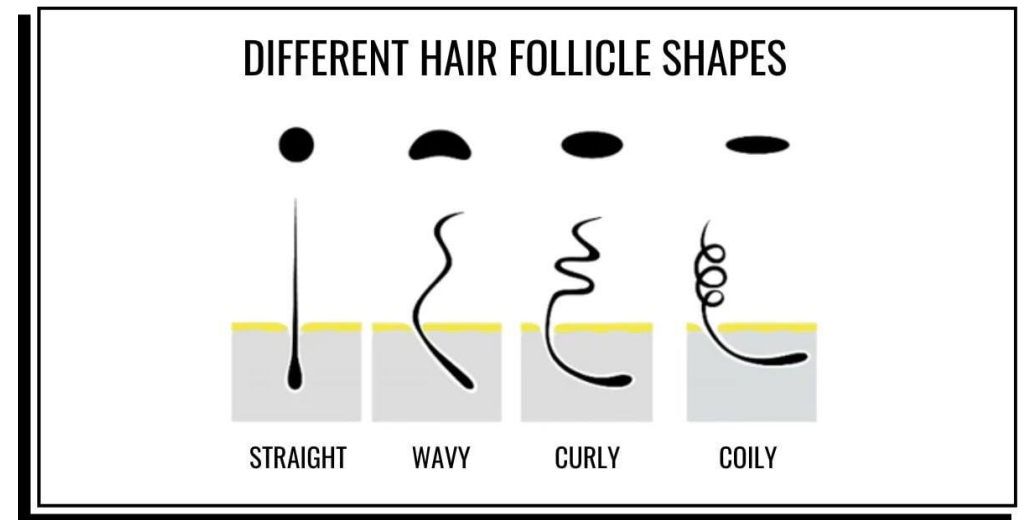


- Scalp massage increases blood flow to the follicles
- Promotes nutrient delivery and may support hair growth
- Helps relieve stress and tension in clients
- Boosts the effectiveness of products like oils and serums
- Use a gentle, confident technique — pressure, rhythm, and consistency matter

- **Day 4 Part 3**
- **Discussion Prompts:**
 - Have you tried incorporating massage into your services?
 - How does a client typically respond to a good scalp massage?
 - What makes a massage feel professional instead of awkward?

Day 5 – Part 1: Hair Types, Shapes & Ethnic Variation

- Asian hair: thick, straight strands with round follicles and high density
- European hair: oval follicles; varied diameter and wave patterns
- Afro-textured hair: tight curls, flat elliptical follicles, fragile and dry
- Follicle shape determines curl pattern, elasticity, and strength
- Hair type affects product absorption, moisture retention, and breakage risk



- **Day 5 Part 1**
- **Discussion Prompts:**
 - Which hair type do you find most challenging to treat or style?
 - How would you adapt techniques for different hair types, such as fragile and dense hair?
 - Why is it essential to recognise ethnic variations in hair care?
 - How do you adapt products for different hair types?
 - What texture or curl pattern do you work with most?

Day 5 – Part 2: Growth Patterns & Tension Zones

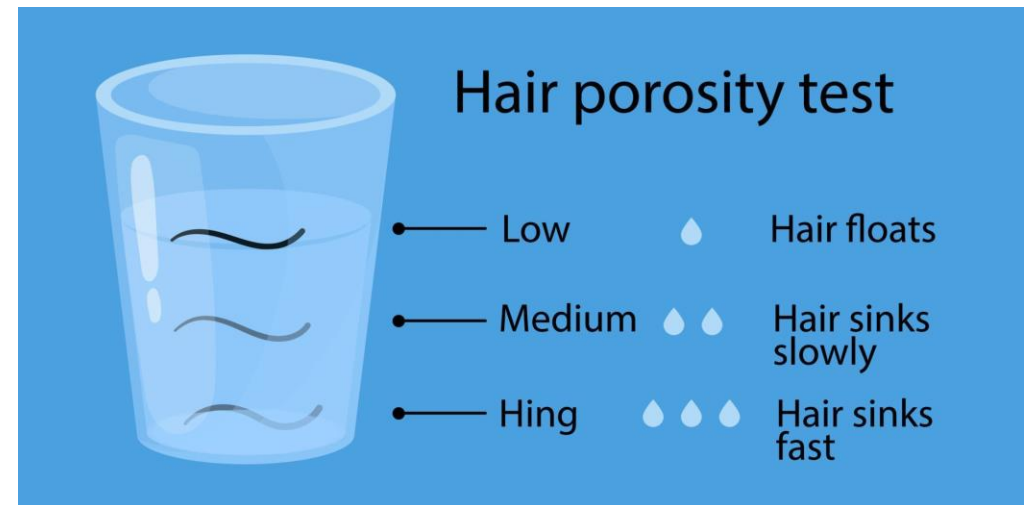


- Scalp patterns: crowns, cowlicks, and whorls affect hair direction and styling
- Growth direction impacts tension, partings, and cut results
- Tension zones: temporal, crown, and occipital areas prone to traction stress
- Tight styles (e.g. braids, weaves) can cause tension damage
- Use mapping and gentle styling to reduce breakage risks
- Protective styles must balance aesthetics with long-term scalp health

- **Day 5 Part 2**
- **Discussion Prompts:**
 - How do you adapt styling for clients with strong growth patterns?
 - Where do you commonly notice tension-related thinning?
 - How do you educate clients on safe styling habits?
 - How can we style tighter hair types without causing damage?

Day 5 – Part 3: Analysing Texture, Porosity & Elasticity

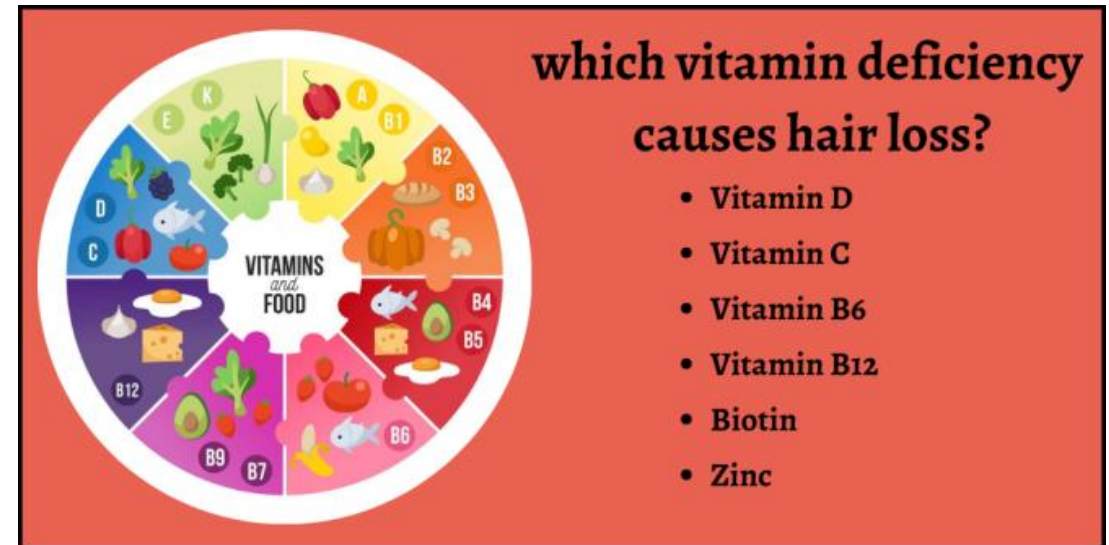
- Porosity test shows how hair absorbs and retains moisture: low, normal, or high
- Elasticity test reveals strength and flexibility — snap vs stretch = moisture vs protein
- High porosity hair absorbs quickly but dries fast — often damaged or over-processed
- Product build-up can distort results — clarify before testing
- Shrinkage, frizz, or snapback offer visible cues for client education



- **Day 5 Part 3**
- **Discussion Prompts:**
 - How do you check for porosity or elasticity during a consultation?
 - What signs help you decide if hair lacks protein or moisture?
 - How do you explain moisture vs protein needs to clients?
 - What small tests can help you understand hair condition

Day 6 – Part 1: Key Nutrients for Hair Growth

- Hair is non-essential tissue — signs of nutrient deficiency appear early
- Iron, Zinc, Vitamin D3, B12: vital for healthy follicle function and cell turnover
- Protein supports keratin synthesis — low intake = excess shedding
- Biotin only benefits hair if a deficiency is present — not a miracle cure
- Hydration and antioxidant-rich foods help maintain the scalp barrier and reduce inflammation
- Protein is essential — hair is made of keratin
- Crash dieting or dehydration can show up as dullness or shedding
- We can't diagnose, but we can raise awareness and recommend speaking to a GP

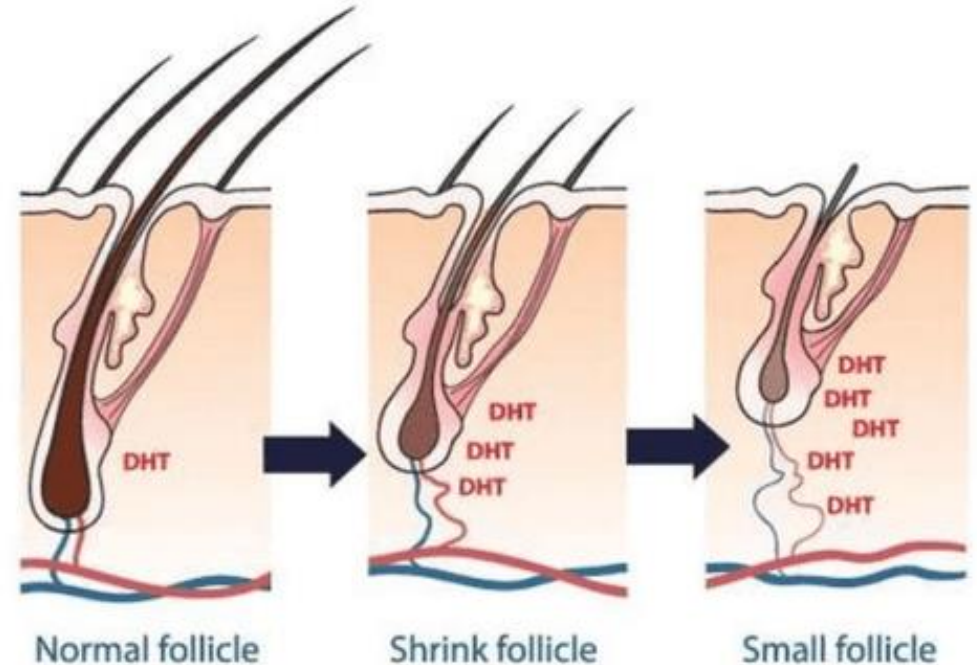


- **Day 6 Part 1**
- **Discussion Prompts:**
 - Have you noticed how diet affects your own hair?
 - How can we discuss nutrition without stepping outside our role?
 - What signs suggest a client may have a nutritional deficiency?
 - How could you ask about nutrition without overstepping?
 - What signs suggest internal stress rather than external damage

Day 6 – Part 2: Hormonal Imbalances & Hair Shedding

- DHT (a form of testosterone) contributes to male/female pattern hair loss
- Menopause: drop in oestrogen leads to follicle shrinkage and thinning
- PCOS: high androgens and insulin resistance can trigger shedding
- Postpartum hair loss = Telogen Effluvium due to hormone fluctuations
- Thyroid disorders slow regrowth and alter hair texture and density

The Effects of DHT on Hair Growth



- **Day 6 Part 2**
- **Discussion Prompts:**
 - Have you seen hormonal changes affect your own or clients' hair?
 - How would you explain Telogen Effluvium to a worried client?
 - What are respectful ways to suggest someone might need medical tests?

Day 6 – Part 3: The Impact of Stress, Sleep & Habits

- High cortisol from chronic stress can push hair into resting (telogen) phase
- Poor sleep disrupts scalp repair, growth signals, and barrier recovery
- Gut health and inflammation may trigger autoimmune-related hair loss
- Smoking, alcohol, and crash diets reduce blood flow and nutrient supply to follicles
- Stylists can help spot lifestyle-related triggers during consultation



2009



2012

- **Day 6 Part 3**
- **Discussion Prompts:**
 - Have you seen links between stress and sudden hair changes in clients?
 - What lifestyle habits would you gently ask about during a consultation?
 - How can we stay supportive without sounding judgmental?

Day 7 – Part 1: Building Trust Through Consultation

- Consultation goals: gather info, educate the client, build reassurance
- Tone, posture, and empathy establish immediate rapport and safety
- Key intake info: health history, lifestyle, medications, allergies
- Open vs closed questions: use open ones to explore and build trust
- Respect client privacy — know GDPR basics around data handling



- **Day 7 Part 1**
- **Discussion Prompts:**
 - What questions help you build trust during a first appointment?
 - How can tone and body language affect a nervous client?
 - Why is health and lifestyle information important for scalp care?

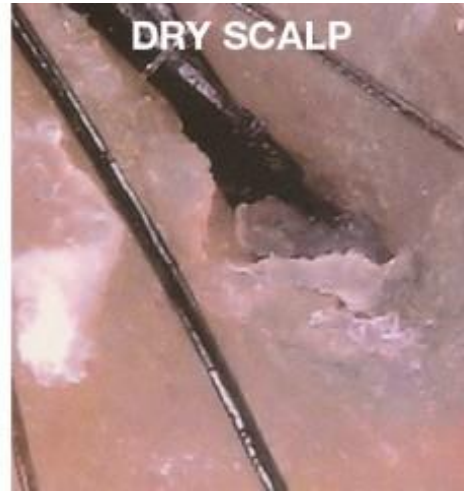


Day 7 – Part 2: Allergy Testing & Observation Tools

- Always follow brand guidelines for allergy testing (e.g., 48-hour patch test)
- Use Colourstart if trained and permitted — not a replacement for GP testing
- Look for signs: redness, flaking, sores — don't diagnose, observe
- Use a dermatoscope or phone camera (if appropriate) for monitoring
- Keep simple notes: what you see, what you recommend

- **Discussion Prompts:**

- How do you explain the importance of allergy testing to clients?
- Why is it helpful to visually track a client's scalp changes?
- What do you look for during a scalp check before colouring?
- When would you avoid working on someone's scalp?
- What language helps you stay safe legally and professionally?



Day 7 – Part 3: Reviewing Results & Maintaining Engagement

- Review progress using consistent photos, notes, and consultation forms
- Set clear timelines — hair/scalp improvements often take weeks to months
- Stay honest but supportive: manage expectations kindly
- Address sensitive issues (stress, alopecia) with empathy and discretion
- Know when to refer again or adapt the current treatment plan

- **Day 7 Part 3**
- **Discussion Prompts:**
 - How do you handle clients who expect instant results?
 - What's your strategy when a treatment isn't working?
 - When should a client be referred again to a medical expert?
 - What helps you manage client expectations over time?
 - How can you stay supportive when clients are emotional?



Day 8 – Part 1: Becoming a 'Scalp-Aware' Hair Professional

- Use trichology knowledge to build trust — not to diagnose
- Spot common scalp issues and explain concerns using safe language
- Document client concerns clearly without making clinical claims
- Refer to medical experts when issues fall outside your role
- Offer scalp-focused consultations to elevate client experience

- **Day 8 Part 1**
- **Discussion Prompts:**
 - What language would you use to refer a client without overstepping?
 - How could a scalp-care focus help you stand out as a hairdresser?
 - Have you ever treated someone who really needed to see a specialist?

Day 8 – Part 2: Creating Simple Scalp Care Services

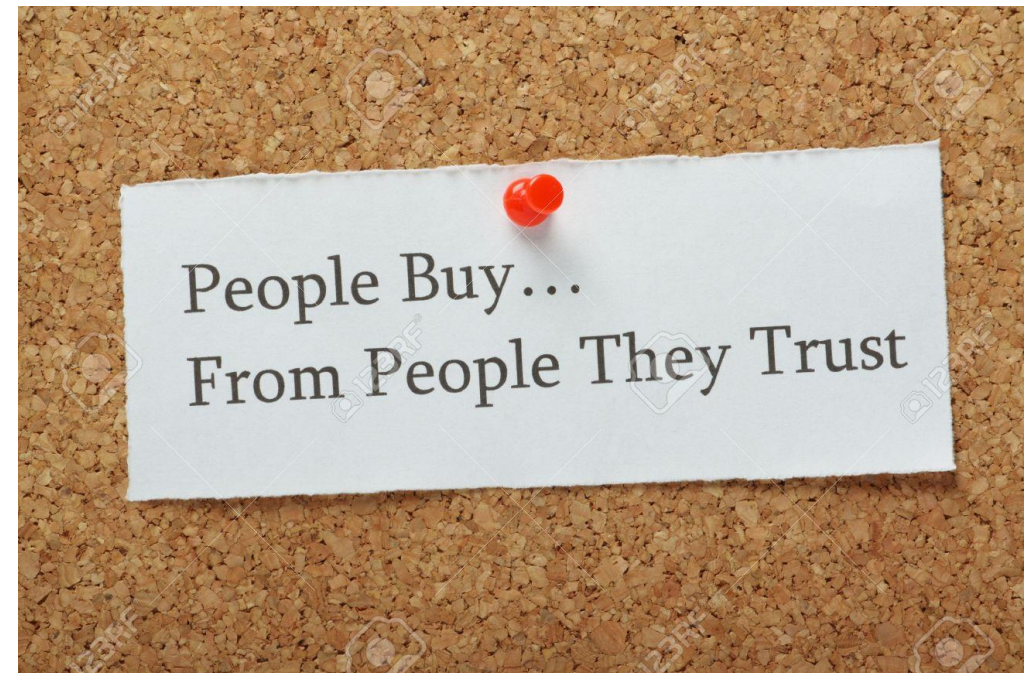
- Offer cleanse, exfoliate, massage, and moisturise treatments safely
- Use non-medical tools: steamer, scrubs, calming serums, natural oils
- Create short scalp rituals as luxury add-ons or standalone services
- Price based on time, product use, and value — not 'treatment claims'
- Know when NOT to treat: open wounds, active infections, unknown reactions



- **Day 8 Part 2**
- **Discussion Prompts:**
 - Which treatments do you already offer that could benefit the scalp?
 - What's one simple scalp ritual you could create for your clients?
 - How can you make this feel luxurious without crossing clinical boundaries?

Day 8 – Part 3: Building Confidence & Conversations

- Use consultation forms to naturally introduce scalp discussions
- Educate clients with simple terms: build-up, flaking, dryness, shedding
- Take photos to show progress — builds trust and client satisfaction
- Promote scalp services with ethical wording and disclaimers
- Continue learning: partner with trichologists or refer when needed



- **Day 8 Part 3**
- **Discussion Prompts:**
 - What stops professionals from offering scalp care confidently?
 - How can you promote these services without sounding medical?
 - Would you consider collaborating with a trichologist in future?